

Reusable Medical Device Reprocessing and Sterilization

Policy Number	POL-IC-027	Owner / Department	Sterile Processing / Infection Prevention & Control
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Last Revised	2026-03-20	Next Review	2028-03-20
Approved By	Infection Prevention & Control Committee	Applies To	Sterile Processing, perioperative and procedural areas, endoscopy, and any area that reprocesses reusable devices

1. Purpose

To ensure reusable medical devices are cleaned, disinfected, and sterilized correctly and consistently with the manufacturer's instructions for use (IFU) and the Spaulding classification, consistent with CDC, AAMI, and AORN standards.

2. Definitions

- **Cleaning** — removal of visible soil and organic material; a prerequisite for all disinfection and sterilization.
- **High-level disinfection (HLD)** — a process that kills all microorganisms except large numbers of bacterial spores.
- **Sterilization** — a validated process that destroys all microbial life, including spores.
- **IFU** — the device manufacturer's instructions for use, which govern the validated reprocessing method and are followed exactly.

3. Spaulding Classification

The required level of reprocessing is determined by how the device contacts the patient. When the IFU and this table conflict, follow the stricter requirement and consult Infection Prevention.

Category	Contact	Minimum reprocessing
Critical	Enters sterile tissue or the vascular system.	Sterilization (e.g., surgical instruments, implants).
Semi-critical	Contacts mucous membranes or non-intact skin.	High-level disinfection (e.g., flexible endoscopes, laryngoscope blades).
Non-critical	Contacts intact skin only.	Low- or intermediate-level disinfection (e.g., BP cuffs, stethoscopes).

IMPORTANT — Every sterilizer load containing an implant must include a biological indicator, and the implant is quarantined and not released until the biological indicator result is confirmed negative. Never release an implant on the basis of chemical indicators alone.

4. Procedure

1. Pre-clean at the point of use to prevent soil from drying; transport soiled items in a closed, labeled container to the decontamination area.
2. Clean per IFU, then inspect for cleanliness and function before disinfection or sterilization.
3. Monitor every sterilizer load with mechanical, chemical, and biological indicators; run a biological indicator at least daily (and in every implant load).
4. Document HLD cycles (solution, concentration test, contact time, temperature) and endoscope reprocessing, including storage; maintain lot/load traceability to the patient.

5. Severity & Reporting

Reprocessing failures are classified under POL-RM-013. Use of an improperly reprocessed critical or semi-critical device on a patient, or a positive biological indicator requiring patient notification/look-back, is escalated as SEV-1 (or SEV-2 if no infection results and only monitoring is needed) — for SEV-1 notify the attending provider, Nursing Supervisor, Risk Management, Infection Prevention, and the Administrator-on-Call immediately and within 1 hour. A documentation or cycle-monitoring lapse without patient exposure is SEV-3, and a failed load caught before any device is used is a Near Miss; both are reported to Infection Prevention within 24 hours. A device-related injury may also be reportable to the FDA via MedWatch by Risk Management.

6. Documentation Requirements

- Load records with mechanical, chemical, and biological indicator results, and lot/load-to-patient traceability.
- HLD logs (solution potency test, contact time, temperature) and endoscope reprocessing and storage records.
- For any failure: devices and patients affected, look-back actions, assigned severity, FDA/public-health notification if applicable, and all notifications.

7. References

- CDC, Guideline for Disinfection and Sterilization in Healthcare Facilities (2008, with updates); Spaulding classification.
- ANSI/AAMI ST79 (steam sterilization) and ST91 (flexible endoscope reprocessing); AORN perioperative guidelines.
- The Joint Commission IC standards; CMS Conditions of Participation, 42 CFR 482.42.
- Related: POL-RM-013 (severity & reporting), POL-IC-020 (hand hygiene).